

IDSP Key Activities & Achievements

1. Objectives

To strengthen/maintain decentralized laboratory-based IT enabled disease surveillance system for epidemic-prone diseases to monitor disease trends and to detect and respond to outbreaks in early rising phase through trained Rapid Response Team (RRTs).

2. Programme Components

- Integration and decentralization of surveillance activities through the establishment of surveillance units at Centre, State and District level.
- Human Resource Development – Training of State Surveillance Officers, District Surveillance Officers, Rapid Response Team and other Medical and Paramedical staff on principles of disease surveillance.
- Use of Information Communication Technology for collection, collation, compilation, analysis and dissemination of data.
- Strengthening of public health laboratories.
- Inter sectoral Co-ordination for zoonotic diseases.

3. Data Management

Under IDSP, data on epidemic-prone diseases is collected on a real-time basis. Information is collected through three specified reporting formats, namely **“S” (Suspected Cases)**, **“P” (Presumptive Cases)** and **“L” (laboratory Confirmed Cases)**, which are filled by Health Workers, Clinicians and Laboratory staff respectively.

A Total of 184,895 Reporting Units (RUs) are covered under Syndromic Surveillance (S Form) of which 1,42,888 (77%) reported, In Presumptive Surveillance (P form) Total of 43,180 RUs, of which 35,003(81%) reported, In laboratory-confirmed surveillance (L Form) Total of 36,414 RUs, of which 29,949 (82%) reported in the Year 2025(January- December) on IDSP-IHIP Portal.

Whenever there is a rising trend of illnesses in any area, it is investigated by the Rapid Response Teams (RRTs) to diagnose and control the outbreak. Data analysis and response actions are undertaken by the respective State and District Surveillance Units.

- **Outbreak Surveillance and Response**

CSU, IDSP receives disease outbreak reports from the States/UTs on a weekly basis. Even NIL weekly reporting is mandated and compilation of disease outbreaks/alerts is carried out on a weekly basis. On an average, 40 outbreaks are reported to CSU every week.

A Total 554 outbreaks in 2020, 728 outbreaks in 2021, 1027 outbreaks in 2022, 1862 outbreaks in 2023, 3020 outbreaks in 2024 and 2285 outbreaks in 2025.

In the Year 2026, a total of 411 outbreaks have been reported till the 10th week (ending on 08/03/2026) The majority of outbreaks reported were of Acute Diarrhoeal Disease (ADD), Food poisoning, Chickenpox, Measles, Dengue, Chikungunya, Malaria & Cholera.

4. Media Scanning and Verification Cell

Media scanning is an important component of surveillance to detect early warning signals. The Media Scanning and Verification Cell receive an average of 5–6 media alerts of unusual health events on a daily basis, which are subsequently detected and verified. A Total of 14,533 media alerts have been scanned till 31st March 2026 since the establishment of the cell in 2008. The process of establishing Media Scanning and Verification Cells in all States and UTs is being undertaken.

5. IHIP Implementation

On average 78.31%, 82.01% and 83.40% Reporting Units have reported in S (Syndromic) form, P (Presumptive) form and L (Laboratory Confirmed) form respectively under IDSP-IHIP Portal (1st April 2025 to 31st March 2026).

6. Public Health Emergency Operation Centre (PHEOC)

was activated in response to the multiple public health events: Acute Diarrhoeal Disease in Jajpur, Odisha in June, 2025, Nipah virus in Kerala in July 2025, Guillain-Barré syndrome (GBS), Mandsaur Madhya Pradesh in August 2025, Acute Kidney Injury outbreak in Chhindwara, Madhya Pradesh in October 2025, 'Vishanu Yuddh Abhyaas' focusing on Crimean-Congo Haemorrhagic Fever (CCHF) in Khandwa, Madhya Pradesh, conducted from November 4-6, 2025, Acute Diarrhoeal Disease in Indore, Madhya Pradesh in December 2025, Nipah virus in West Bengal in January, 2026, Guillain-Barré syndrome (GBS), Neemuch, Madhya Pradesh in January, 2026 and Hepatitis, Palwal Haryana in February, 2026.

7. Lab Network

The Integrated Disease Surveillance Programme (IDSP) lab network is structured as a two-pronged system, focusing on basic routine laboratory testing and outbreak investigations at

District Public Health Laboratories (DPHLs), and referral testing of outbreak-related samples at State Referral Laboratories (SRLs).

- **District Public Health Laboratories (DPHLs)-** Strengthening of public health labs is an important component of IDSP that focuses on developing lab capacity in India. Till March 2026, 441 district public Health labs in 36 States/ UTs have been approved for strengthening, out of which 397 (90%) labs have performed as per tests recommended by IDSP.
- **State Referral Labs-** Existing functional laboratories in Govt Medical college have been strengthen to act as State Referral Laboratory for confirmatory testing & outbreak investigations. At the state level, the IDSP-SRL network is operational across **27 States/UTs with 163 sanctioned SRLs.**

8. Metropolitan Surveillance Units (MSUs)

The Metropolitan Surveillance Unit (MSU) is a central sector scheme under the Pradhan Mantri Ayushman Bharat Health Infrastructure Mission (PM-ABHIM) to address critical gaps in surveillance through establishment of dedicated surveillance units in 20 major cities. Presently there are 17 operational MSUs at Mumbai, Ahmedabad, Bhubaneswar, Nagpur, Chennai, Bengaluru, Pune, Thane, Patna, Hyderabad, Gurugram, Guntur, Shimla, Varanasi, Agra, Jaipur and Guwahati.

The Key objectives: To Serve as Surveillance Hub, Facilitate planning and coordination of response to disease outbreaks & Capacity Building.

9. Vaccine Preventable Diseases Surveillance

The Government of India, through the Ministry of Health & Family Welfare and the National Centre for Disease Control (NCDC), has established a robust Vaccine Preventable Disease (VPD) surveillance system as a cornerstone of national health security, enabling evidence- based immunization policy, early outbreak detection, and rapid response across the country. This system covers six priority diseases—polio (AFP), measles, rubella, diphtheria, pertussis, and neonatal tetanus. Through a three-phased transition of VPD surveillance functions from WHO-NPSN to IDSP–NCDC, with full national ownership targeted by 31st March 2027. Two phases of this transition have taken place, with requisite trainings of the district and state officials:

- **In Phase-1:** 246 districts and 12 complete states were transitioned on August 31, 2025.
- **In Phase 2:** 171 districts and 3 states were completely transitioned on March 31, 2026.
- **In Phase 3:** 388 districts are planned to be transitioned by March 31, 2027.

At the state level, State Surveillance Units (SSUs) under respective State Health Departments will own day-to-day surveillance operations, with the Central Surveillance Unit (CSU) at IDSP-NCDC providing national oversight, technical guidance, and programme monitoring. The

Immunization Division of MoHFW will remain a key partner, particularly for disease elimination goals and policy decisions.

10. NJORT

The National Joint Outbreak Response Team (NJORT), deployed for outbreak investigation, comprises of experts from NCDC, MoHFW, DAHD, MoFAH&D, ICMR, MoH&FW, ICAR, MoA&FW, MoEF&CC, FSSAI, MoH&FW and Ministry of Jal Shakti, who work collaboratively with local authorities to comprehensively investigate the outbreak, provide specific recommendations, based on the findings and analysis of data from the investigation for public health action. In 2026, NJORT was deployed in Madhya Pradesh (Neemuch) in January for Guillain-Barré Syndrome (GBS) and in West Bengal for Nipah virus outbreaks.

11. PHDEM

The National Centre for Disease Control, National Institute of Disaster Management, and Centers for Disease Control and Prevention India are implementing CB-PHEDM Training of Trainers (ToT) programmes across India in coordination with various state governments. These trainings are conducted in a structured manner across different tiers and batches to strengthen community-based preparedness. The primary objective of the training is to build capacity for risk identification, participatory planning, and effective community-led response to public health emergencies and disasters. The programme has engaged officers from key line departments—including IDSP, Animal Husbandry, Medical Colleges, SDRF, ICDS, PHED, Education, Wildlife, Red Cross, Civil Defence, and Factory and Boilers—with a total of 1,078 participants trained till date.